Claimflow — Digital Integration Overview and Support Document

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1. How Insurance Claims Actually Work in India Today

1.1 Current ecosystem players

Hospitals (Network providers) ↔ TPAs (Third Party Administrators) ↔ Insurance Companies ↔ Policyholders

Two claim pathways:

A. Cashless (Pre-authorization) flow:

1. Patient admitted to network hospital
2. Hospital TPA desk submits **pre-authorization request** to TPA/insurer with:
    - Policy number, patient ID, doctor's notes
    - Provisional diagnosis, treatment plan
    - Estimated costs
3. TPA/insurer verifies:
    - Policy validity, sum insured, waiting periods
    - Exclusions, sub-limits, room rent caps
4. Approval issued (full/partial/rejected) with authorization number
5. Treatment proceeds
6. Hospital submits **final bill + discharge summary + all documents** for settlement
7. TPA/insurer settles directly with hospital (≤3 hours from final bill submission per IRDAI norms)[1]

B. Reimbursement flow:[3]

1. Patient pays hospital out-of-pocket
2. Patient submits claim documents to TPA/insurer:
    - Filled claim form

    - Original bills, discharge summary, prescriptions
    - Investigation reports, case papers
3. TPA verifies and processes
4. Insurer reimburses patient

1.2 Pain points your SaaS should solve

For hospitals:[4][5]

- Multiple TPAs/insurers = fragmented workflows
- Delayed claim approvals (hours to days)
- High rejection rates due to incomplete documentation
- Cash flow stress (pending claims for weeks/months)
- Manual paperwork and follow-ups

For insurers/TPAs:[5][1]

- Incomplete/illegible documents from hospitals
- Fraud detection is manual and reactive
- No real-time visibility into treatment progress
- Delayed final bill submission
- High operational costs (scrutiny, follow-ups)

2. Your SaaS Platform Architecture

2.1 Core value proposition

"Single API integration for hospitals to connect with ALL insurers/TPAs + AI-powered fraud prevention + real-time claim lifecycle tracking"

Think of it as **"Stripe for health insurance claims in India"** – one integration, multiple payers, instant settlements.

2.2 Platform design (Hospital → Your Platform → Insurers)

```
┌──────────────────────────────────────────────────────────────────────────────────────────────────────────┐
│  HOSPITAL LAYER (Your Integration Points)                                                                 │
│   ├── Hospital Management System (HMS) API integration                                                    │
│   ├── Manual upload portal (for smaller hospitals)                                                        │
│   └── ABDM integration (for EHR data pull)                                                                │
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┌──────────────────────────────────────────────────────────────────────────────────────────────────────────┐
│  YOUR PLATFORM (Central Intelligence Layer)                                                               │
│                                                                                                           │
│  1. Document Processing Engine                                                                            │
│      ├── OCR for handwritten notes (your strength!)                                                       │
│      ├── Auto-extraction of MCI-compliant fields                                                          │
│      ├── Structured data validation (ICD-10, PCS codes)                                                   │
│      └── Completeness check against IRDAI standards                                                       │
│                                                                                                           │
│  2. Fraud Detection & Risk Scoring Engine (AI/ML)                                                         │
│      ├── Real-time anomaly detection                                                                      │
│      ├── Pattern analysis across claims                                                                   │
│      ├── Network analysis (provider-patient relationships)                                                │
│      └── Predictive risk scoring                                                                          │
│                                                                                                           │
│  3. Smart Routing & Workflow Engine                                                                       │
│      ├── Auto-route to correct TPA/insurer                                                                │
│      ├── Pre-auth and settlement workflow orchestration                                                   │
│      ├── Real-time status tracking                                                                        │
│      └── Auto-follow-ups and alerts                                                                       │
│                                                                                                           │
│  4. Data Standardization Layer                                                                            │
│      ├── Convert to EHR Standards format                                                                  │
│      ├── ABDM-compliant data structures                                                                   │
│      ├── FHIR R4 bundles for interoperability                                                             │
│      └── Standard nomenclatures (ROHINI, ICD-10, PCS)                                                     │
│                                                                                                           │
│  5. Analytics & Compliance Dashboard                                                                      │
│      ├── Hospital: pending claims, rejection analytics                                                    │
│      ├── Insurer: claim queue, fraud alerts, TAT metrics                                                  │
```

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│      └── Regulatory reporting (IRDAI compliance)             │
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                                ↓
┌──────────────────────────────────────────────────────────────────────────────────────────────────────────┐
│  PAYER LAYER (Insurance Companies & TPAs)                   │
│   ├── API integration with major TPAs/insurers               │
│   ├── Standard pre-auth and settlement APIs                  │
│   └── Fraud alert webhooks                                   │
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```

3. How to Reduce Friction & Speed Up Claims

3.1 Pre-authorization acceleration

Current problem: Manual pre-auth takes hours to days[2]

Your solution:

- **Instant completeness check:** Before submission, auto-validate all required documents against insurer-specific checklists
- **AI-powered cost estimation:** Compare treatment plan with historical approved cases for same diagnosis/procedure → predict approval amount
- **Auto-missing document alerts:** Flag missing items in real-time to hospital
- **Smart document routing:** Send structured data + original scans simultaneously
- **Real-time status API:** Hospitals see approval status on your dashboard within minutes

Target: Reduce pre-auth approval time from average 2-4 hours to **<30 minutes**[2]

3.2 Final settlement acceleration

Current problem: Final bills take 3-7 days (or weeks) to settle[5][1]

Your solution:

- **Progressive documentation:** Upload investigation reports, procedure notes, nursing notes as they happen (not in one batch at discharge)
- **Pre-discharge reconciliation:** Auto-generate itemized bill matching approved pre-auth + flag variance
- **Discharge summary auto-generation:** Your OCR + LLM generates structured discharge summary from case notes in MCI format
- **Instant submission:** Final packet auto-submitted to insurer at discharge
- **3-hour settlement guarantee:** Per IRDAI norms, push insurers to settle within 3 hours of final bill[1]

Target: Reduce settlement time from 7-30 days to **3 hours to 2 days**

3.3 Rejection prevention

Current problem: 20-30% claims require clarification or get partially rejected[5]

Your solution:

- **Pre-submission audit engine:**
  - Check policy validity, waiting periods, exclusions
  - Flag room rent caps, disease sub-limits
  - Verify documents against insurer-specific requirements
- **Smart alerts to hospital:** "Patient has diabetes (pre-existing), ensure declaration present in form"
- **Auto-correction suggestions:** "Investigation report missing radiologist signature"

Target: Reduce rejection rate from 20-30% to **<5%**

4. Fraud Detection & Prevention (Critical for Insurer Buy-in)

4.1 Why fraud detection is your competitive moat

IRDAI just released new Fraud Monitoring Framework Guidelines (effective April 2026) requiring:[8][9][10]

- **Zero-tolerance fraud policies**

- **Board-level Fraud Monitoring Committees**
- **Fraud Monitoring Units (FMUs)** in every insurer
- **Industry-wide fraud data sharing** via Insurance Information Bureau (IIB)
- **Advanced analytics and AI/ML for fraud detection**
- **Report all frauds >₹1 crore within 30 days**

Insurers desperately need tech solutions to comply. You're perfectly positioned.

4.2 Types of healthcare fraud in India

A. Provider (Hospital) fraud:

- Billing for services not rendered
- Upcoding (billing higher procedure codes)
- Unbundling (splitting one procedure into multiple billable items)
- Duplicate billing
- Phantom patients
- Inflated bills (collusion with patients)

B. Patient (Policyholder) fraud:

- Staged accidents/illnesses
- Fake documentation
- Non-disclosure of pre-existing conditions
- Duplicate claims to multiple insurers

C. Collusion fraud (Provider + Patient):

- Planned hospitalizations
- Inflated treatment costs
- Splitting/sharing claim amounts

D. Internal fraud (Staff):

- Ghost claims
- Data manipulation

- Identity theft

4.3 Your AI/ML fraud detection engine (real-time scoring)

Build a multi-layered fraud detection system:

Layer 1: Rule-based red flags (instant alerts)

```
# Examples of rule-based red flags
- Claim amount >3x historical median for same diagnosis/procedure
- Admission within 30 days of policy purchase (waiting period abuse)
- Same patient, multiple claims in <90 days
- Hospital billing pattern deviates >50% from peer hospitals
- Treatment duration outlier (too short/long for diagnosis)
- Missing key documents (no investigation reports for surgery)
- Bill submitted >6 months after discharge
- High-value procedures on weekends/holidays (staged care)
```

Layer 2: ML anomaly detection models

Use supervised + unsupervised learning:

A. Supervised models (labeled fraud cases):

- **Gradient Boosting (XGBoost/LightGBM)** – best for tabular healthcare data
- **Neural networks** for complex pattern recognition

Features to engineer:

- Patient: age, gender, policy tenure, claim history frequency, gap between claims
- Hospital: claim volume, average claim amount, rejection rate, specialty mix
- Claim: diagnosis (ICD-10), procedures (PCS), item-level billing patterns, total amount, length of stay, doctor specialty
- Temporal: admission day/time, claim submission delay, investigation dates
- Network: patient-doctor-hospital relationship graph (frequency of interactions)

Target: Flag fraudulent claims with **>85% precision, <10% false positives**[13]

B. Unsupervised models (unknown fraud patterns):

- **Isolation Forest** – detects anomalies in high-dimensional claim data[12]
- **Support Vector Machines (One-class SVM)** – identifies outliers
- **Autoencoders** – learns "normal" claim patterns, flags deviations
- **Graph neural networks** – detects collusion networks (hospital-patient-doctor rings)

Layer 3: Network analysis (fraud rings)[14][15]

Build a knowledge graph:

```
Nodes: Hospitals, Doctors, Patients, Pharmacies, Diagnostic centers
Edges: Claims (with metadata: amount, date, diagnosis)

Detect:
- High-frequency patient-hospital pairs (staged admissions)
- Doctor-patient clusters with unusual claim patterns
- Hospital-pharmacy networks with inflated billing
- Temporal patterns (coordinated claims in short windows)
```

Use community detection algorithms (Louvain, Label Propagation) to identify fraud rings.

Layer 4: Generative AI for document fraud[13]

Detect fake/altered documents:

- **Computer vision models** to detect:
  - Photoshopped bills/prescriptions
  - Inconsistent fonts/formatting (copy-paste fraud)
  - Forged signatures
  - Altered dates/amounts
- **LLM-based semantic analysis:**
  - Does discharge summary match investigation reports?
  - Are medication prescriptions clinically consistent with diagnosis?
  - Does operative note align with pre-auth treatment plan?

Example check:

```
Pre-auth: "Laparoscopic cholecystectomy"
Final bill: Includes "open surgery" charges
→ Flag: Upcoding detected
```

Layer 5: Real-time risk scoring

For every claim, generate a fraud risk score (0-100):

```
Low risk (0-30): Auto-approve, fast-track settlement
Medium risk (31-70): Flag for human review, request additional docs
High risk (71-100): Hold payment, trigger investigation, alert FMU
```

Provide explainability:

- "Flagged due to: (1) Claim amount 4.2x median, (2) Patient-hospital pair submitted 3 claims in 60 days, (3) Missing post-op X-ray"

4.4 Data sources for fraud detection

You'll need access to:

1. **Your platform's historical claims data** (build proprietary dataset)
2. **Insurance Information Bureau (IIB) fraud repository** (once IRDAI opens access)[8][10]
3. **Public datasets:**
    - IRDAI complaint data
    - NABH-accredited hospital list
    - ABDM provider registry (ROHINI IDs)[16]
4. **External APIs:**
    - Doctor registration verification (State Medical Councils)
    - Hospital empanelment status (PM-JAY, state schemes)

4.5 Fraud prevention workflows

A. Pre-authorization stage:

- Run fraud risk score before approval

- If high-risk → request video consultation proof, additional medical records
- If medium-risk → approve with conditions (e.g., require post-op scans)

B. Settlement stage:

- Compare final bill with pre-auth line-by-line
- Flag new items not in approved list
- Verify investigation reports match billed procedures
- Cross-check pharmacy bills against prescription

C. Post-settlement:

- Continuous monitoring for patterns (same patient filing claims across multiple insurers on your platform)
- Alert insurers of blacklisted entities (via IIB)

5. Technical Architecture & Standards Compliance

5.1 Hospital integration options

Option A: API integration (large hospitals with HMS)

- Integrate with popular HMS vendors in India:
  - **MocDoc, Vikas HIS, Sigma HIS, Remedy HMS**
  - **eClinicalWorks, Practo Ray, Cloudphysician**
- Use HL7 FHIR R4 APIs for data exchange[19]

Option B: Manual portal (small nursing homes, AYUSH centers)

- Web dashboard for staff to upload documents
- Mobile app for bedside data entry
- Your OCR engine auto-extracts data from uploads

Option C: ABDM integration (future-proof)

- Register as Healthcare Information Provider (HIP) with ABDM

- Pull patient EHR data via consent-based APIs
- Auto-populate claim forms with ABHA (Ayushman Bharat Health Account) linked data
- Push claim outcomes back to patient's health locker

Use Eka.care ABDM Connect or similar SDK for plug-and-play ABDM APIs[21][20]

5.2 Data standards (critical for interoperability)

You MUST implement:

Standard	Use Case	Compliance
EHR Standards for India (MoHFW 2016)	Discharge summary format, MDS fields	Mandatory for ABDM
HL7 FHIR R4	Data exchange bundles	ABDM requirement
ICD-10	Diagnosis coding	Insurer billing, analytics
PCS (Procedure Coding System)	Procedure/surgery coding	Insurer billing
ROHINI codes	Hospital identification	ABDM, standardization[16]
SNOMED-CT	Clinical terminology (optional now, future mandatory)	Semantic interoperability
LOINC	Lab test results coding	Investigation reports

Your platform should auto-code diagnoses/procedures using NLP on free-text clinical notes.

5.3 Security & compliance

You're handling highly sensitive health + financial data:

A. Regulatory compliance:

- **IRDAI regulations:** Data protection, claim settlement timelines[23][24]
- **DISHA (Digital Information Security in Healthcare Act)** – pending but follow draft guidelines
- **IT Act 2000 + SPDI Rules** – personal data protection
- **ABDM Health Data Management Policy** – consent-based data sharing[25]

B. Technical security:

- **End-to-end encryption** (AES-256 for data at rest, TLS 1.3 for transit)
- **ABDM-compliant encryption** for health records exchange[19]
- **Role-based access control (RBAC)** – hospital staff, insurer staff, admins
- **Audit logs** – immutable record of all data access/changes
- **Data anonymization** for analytics/ML training (de-identify PII)

C. Infrastructure:

- **ISO 27001 certification** for ISMS
- **HIPAA-equivalent controls** (though not legally required in India, best practice)
- **India data residency** – host on AWS Mumbai / GCP Mumbai / Azure India regions

5.4 Database design

Core entities:

```
-- Simplified schema

TABLE patients (
  id UUID PRIMARY KEY,
  abha_number VARCHAR, -- ABDM health ID
  name, age, gender, phone, policy_details JSONB
)

TABLE claims (
  id UUID PRIMARY KEY,
  patient_id UUID,
  hospital_id UUID,
  insurer_id UUID,
  claim_type ENUM('cashless', 'reimbursement'),
  status ENUM('pre-auth-submitted', 'approved', 'treatment', 'final-submitted',
'settled', 'rejected'),
  pre_auth_amount DECIMAL,
  final_amount DECIMAL,
  fraud_risk_score INT, -- 0-100
  fraud_flags JSONB,
  created_at, updated_at TIMESTAMP
)
```

```
TABLE documents (
  id UUID PRIMARY KEY,
  claim_id UUID,
  type ENUM('pre-auth-form', 'discharge-summary', 'bills', 'investigation-reports',
'prescription'),
  original_file_url TEXT,
  extracted_data JSONB, -- OCR output
  ocr_confidence FLOAT
)

TABLE fraud_alerts (
  id UUID PRIMARY KEY,
  claim_id UUID,
  rule_triggered VARCHAR,
  risk_score INT,
  details JSONB,
  reviewed BOOLEAN,
  reviewer_notes TEXT
)

TABLE audit_logs (
  id UUID PRIMARY KEY,
  entity_type VARCHAR, -- 'claim', 'document', etc.
  entity_id UUID,
  action VARCHAR, -- 'created', 'viewed', 'updated'
  user_id UUID,
  timestamp TIMESTAMP,
  ip_address VARCHAR
)
```

6. Revenue Model & Go-to-Market

6.1 Who pays? (Both sides)

A. Hospitals pay for:

- **Per-claim processing fee:** ₹50-200 per claim (based on volume tiers)
- **SaaS subscription:** ₹10,000-50,000/month (based on bed capacity) for:
  - Dashboard access
  - Document management
  - Analytics/reporting
- **Premium features:** OCR for handwritten notes, AI fraud pre-check before submission

B. Insurers/TPAs pay for:

- **Fraud detection license:** ₹5-10 lakh/year per TPA/insurer
- **Per-claim fraud scoring API:** ₹10-30 per claim scored
- **Premium analytics:** Benchmarking reports, network optimization insights

Revenue split example (at scale):

- 70% from hospital fees (volume play)
- 30% from insurer fees (high margin, strategic)

6.2 Go-to-market strategy

Phase 1: Hospital onboarding (supply side first)

Why hospitals first?

- They have the pain (cash flow, rejections)
- You solve immediate problem (faster settlements)
- Creates demand-side pressure on insurers ("Why aren't you on this platform?")

Target segments:

1. **NABH-accredited mid-size hospitals (100-300 beds)** in Tier 1/2 cities
   - Already digitized, have HMS
   - High claim volume
   - Pain: dealing with 10-15 different TPAs
2. **PM-JAY empanelled hospitals**
   - Familiar with government digital health initiatives

    - ABDM-ready mindset
3. **Corporate hospital chains** (Apollo, Fortis, Manipal, etc.)
    - Pilot with 1-2 hospitals, scale across network

Pitch: "Single integration, connect to all TPAs. Get paid in 3 days, not 30 days."

Phase 2: Insurer partnerships (demand side)

Approach top 5 TPAs first:

- **Medi Assist** (largest TPA in India)[26][13]
- **Vidal Health** (ICICI Lombard TPA)
- **MD India** (Bajaj Allianz TPA)
- **Paramount Health Services** (Max Bupa TPA)
- **Health India TPA** (United India Insurance)[3]

Value prop:

- "Reduce fraud by 30% with AI"
- "Settle claims 5x faster, reduce operational costs"
- "IRDAI 2025 compliance ready" (fraud monitoring framework)[8]

Phase 3: Network effects

Once you have 50+ hospitals + 3-4 major TPAs:

- **Hospitals:** "All major insurers are here, I must join"
- **Insurers:** "All top hospitals use this, I must integrate"
- **Patients:** Become aware via hospitals ("Your claim is tracked on [YourPlatform]")

6.3 Competitive positioning

Existing players:[27][18][16][17]

- **IHX (Insurance & Healthcare Exchange)** – digital claims platform, ABDM-integrated[16]
- **Remedinet** – TPA automation suite[27]
- **MocDoc, Vikas HIS** – HMS vendors with insurance modules[18][17]

- **Eka.care** – ABDM aggregator with provider tools[20]

Your differentiation:

1. **AI-first fraud detection** (not just workflow automation)
2. **Hospital-centric design** (others are insurer-centric)
3. **OCR + LLM for legacy document digitization** (your unique strength from hospital record digitization experience)
4. **ABDM-native architecture** (future-proof for government mandates)

7. Critical Success Factors

7.1 Metrics to track

Hospital metrics:

- Average pre-auth approval time
- Settlement TAT (turnaround time)
- Claim rejection rate
- Cash flow improvement (days sales outstanding)

Insurer metrics:

- Fraud detection rate (% of fraudulent claims caught)
- False positive rate (legit claims flagged as fraud)
- Processing cost per claim
- Settlement accuracy

Platform metrics:

- Document completeness score (pre-submission)
- API uptime (target: 99.9%)
- OCR accuracy on handwritten notes
- Claims processed per day

7.2 Risk mitigation

Risk 1: Insurer resistance to data sharing

- **Mitigation:** Start with fraud detection as standalone SaaS (don't need claim flow integration initially)
- Position as "your own private fraud detection tool"
- Once they see value, upsell to full integration

Risk 2: Hospital IT integration complexity

- **Mitigation:** Offer manual upload portal as fallback
- Partner with HMS vendors for pre-built connectors
- Provide integration support team

Risk 3: ABDM mandatory compliance delays

- **Mitigation:** Build ABDM-compliant from day 1, but don't make it mandatory for hospitals to use
- Offer as optional enhanced feature

Risk 4: Regulatory changes

- **Mitigation:** Maintain legal/compliance advisory board
- Auto-update platform for IRDAI circular changes
- Position as "compliance-as-a-service"

7.3 Immediate next steps for MVP

Month 1-2: Core platform MVP

1. Build hospital document upload portal (web + mobile)
2. OCR engine for discharge summary extraction (use existing models: Tesseract OCR + Azure Form Recognizer + custom fine-tuning)
3. Basic fraud rule engine (10-15 red flag rules)
4. Manual TPA submission workflow (email/portal)

Month 3-4: Pilot with 2-3 hospitals

1. Onboard 1 NABH hospital (200-300 beds) + 2 smaller hospitals

2. Process 100-200 claims manually to build dataset
3. Validate fraud detection accuracy
4. Measure settlement time improvement

Month 5-6: Insurer API integration

1. Integrate with 1 major TPA (Medi Assist or Vidal)
2. Automate pre-auth submission via API
3. Real-time status tracking
4. Build initial ML fraud model on collected data

Month 7-12: Scale

1. Onboard 20+ hospitals
2. Integrate 3-5 TPAs
3. Launch fraud detection SaaS for insurers
4. ABDM integration pilot

8. Startup-Specific Advice for You

Given your background (AI/ML, healthcare digitization, Supabase/API-heavy architecture):

Your unfair advantages:

1. **Hospital OCR experience** – you already understand document digitization pain
2. **LLM expertise** – use for clinical note summarization, anomaly detection in documents
3. **ABDM familiarity** – you've researched compliance standards

Tech stack recommendation:

- **Backend:** Node.js/Python (FastAPI) on AWS/GCP
- **Database:** Supabase (Postgres) for structured data + S3 for documents
- **OCR:** Azure Form Recognizer (healthcare-trained) + custom fine-tuned models
- **ML:** Python (scikit-learn, XGBoost) → deploy on AWS SageMaker or Modal
- **ABDM:** Eka.care ABDM Connect SDK or OpenABDM[20]

- **Frontend:** React + Capacitor (you're familiar) for hospital mobile app

Fundraising angle:

- **Problem:** ₹15,000+ crore lost to healthcare fraud annually in India (10-15% of claims)[11]
- **Market:** Health insurance claims market growing 15% YoY[11]
- **Timing:** IRDAI 2025 fraud framework = regulatory tailwind[8]
- **Traction path:** 20 hospitals + 1 TPA in 6 months = seed fundable

First customer strategy:

- Target hospitals in **Ahmedabad/Gujarat** first (your location advantage)
- Leverage personal network from hospital digitization work
- Offer 3-month free pilot ("We'll process 100 claims free, you measure settlement time improvement")

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